

How Accountable to the Public Is Funding for Graduate Medical Education? The Case for State Medicaid GME Payments

Thomas M. Henderson, MSPH, MAMC

ABOUT THE AUTHOR

Thomas (Tim) M. Henderson recently retired full time from the Department of Health Administration and Policy, College of Health and Human Services, George Mason University, Fairfax, VA. He continues to work on an adjunct basis and serves as a medical workforce consultant on public policy matters.

Graduate medical education (GME) funding comes from predominantly two public sources: Medicare and Medicaid. In recent years, concerns have been voiced as to whether these GME payments to teaching hospitals and other entities are sufficiently transparent and publicly accountable. Most of these concerns have been directed at the financing and governance of Medicare GME payments. In the past 10 years, two major reports examined this issue.

The main premise of the first report, a 2014 study by the Institute of Medicine (now the National Academy of Medicine), is that Medicare GME payments, and federal funding for GME more broadly, lack a clear purpose. Medicare does not produce enough physicians prepared to practice in the most needed specialties or geographic areas and has insufficient oversight and infrastructure to measure GME program outcomes and reward performance. In terms of accountability, the study found that the stewardship of

the public's investment in GME was critically absent. In particular, any data that teaching hospitals are required to report to the federal government has limited use for program oversight, workforce analysis, or policy development; consequently, the report concluded that most questions about the effectiveness of the Medicare GME program are unanswerable.¹

The second report, a 2018 US General Accounting Office investigation, examined GME spending and related oversight by Medicare and other federal agencies, including what is known about training program costs and what information the federal government collects that documents its investment in GME. The General Accounting Office noted the difficulty the federal government has in identifying GME training costs and how they relate to federal GME funding, including Medicare. The investigation found that because Medicare cost report data are not typically used to

calculate GME payments, federal auditors do not review such data, and there are no federal reporting requirements for how teaching programs use these payments. Ultimately, the General Accounting Office concluded that because the information collected on Medicare GME payments is often incomplete, the federal government lacks sufficient information to comprehensively evaluate their effectiveness.²

Concurrently, the transparency and public accountability of Medicaid GME payments have received little or no scrutiny, in large part because awareness and knowledge of these payments among policymakers and others is limited. I present an overview of Medicaid GME payments and policies, examine some troubling issues regarding the transparency and accountability of these payments, and recommend some federal and state policies to address these issues.

PAYMENTS BY STATE MEDICAID PROGRAMS

Since its inception in the mid-1960s, Medicaid has been an important payor of GME. In the past 20 years alone, state Medicaid programs' GME payments have more than doubled.^{3,4} In 2018, Medicaid GME payments totaled nearly \$5.6 billion—an amount second only to Medicare, the country's largest GME payor. More than half of this amount was paid in the 10 states with the largest number of medical residents, even though Medicaid in two of these states (CA, MA) did not pay for GME (Table 1).⁴

When Medicaid programs are asked why they pay for GME, the most frequent reasons given are to use Medicaid funds to advance state policy goals and to help train the next generation of physicians

who will serve Medicaid beneficiaries.⁴ In particular, 32 states reported in 2015 that they made Medicaid GME payments with the expectation of producing more physicians. Between 1998 and 2009, 10 states consistently stated that they linked Medicaid GME payments to state physician workforce or related policy goals.^{5–10}

How Medicaid is administered and financed provides two key incentives to pay for GME. First, the federal Centers for Medicare and Medicaid Services (CMS) give state Medicaid programs significant flexibility in designing and executing their GME payments, including which professions and which settings and organizations are eligible to receive support for health professions education.¹¹ A number of states have used this flexibility to institute innovative payment approaches. In addition to teaching hospitals, medical schools in three states and resident training sites at

nonhospital patient care settings in two states directly receive Medicaid GME payments. Five states make enhanced payments to individual teaching professionals employed by state university hospitals for services associated with the cost of instructing medical residents. Along with physician residencies, programs that educate graduate nurses and other health professions trainees in 13 states are eligible to receive Medicaid GME payments.⁴

Second, states may receive federal matching funds to help pay for GME under Medicaid.^{4,12,13} In 2018, 16 states reported using contributions from local governments to finance the nonfederal share of their Medicaid supplemental GME payments, which allowed them to claim federal matching funds.⁴

Although not required to do so, 43 states in 2018 made GME payments to teaching hospitals and other entities

under their Medicaid program.^{3,4,12} Of these, 41 states paid for GME under their fee-for-service (FFS) program, and 28 states provided some level of support for GME under managed care.⁴ In 2015, the proportion of Medicaid GME payments made under managed care exceeded the proportion of such payments made under FFS for the first time. In 2018, 52% of all Medicaid GME payments were made under managed care (Table 1).

States distribute Medicaid GME funds differently under FFS and managed care. Under FFS, payments are made to teaching hospitals and other eligible providers through a direct add-on adjustment to the institution's base payment rates and as a federally authorized, lump-sum supplemental payment to help offset any GME costs not reimbursed by the add-on adjustment to base rates.^{4,14} Under managed care,

TABLE 1— Graduate Medical Education (GME) Payments by State Medicaid Programs: United States, 1998–2018

	Year ^a						
	2018	2015	2012	2009	2005	2002	1998
No. of states and DC paying for GME	43	43	43	42	48	48	46
Total GME payments, \$ billions ^b	5.58	4.26	3.87	3.78	3.18	2.70	2.41
% of total GME payments made under FFS and MC, FFS/MC ^c	48/52	39/61	59/41	63/37	75/25	DNC	DNC
% of total GME payments in the 10 states and DC with the largest number of medical residents ^d	55	57	62	60	62	69	DNC

Note. DNC = data not collected; FFS = fee for service; MC = managed care.

Source. Henderson.^{4–10}

^aAll surveys included DC; the 1998 survey also included Puerto Rico. Surveys conducted in 1998, 2002, 2005, 2012, 2015, and 2018 received a 100% response rate. All but one state (AL) responded to the 2009 survey.

^bThe total amount of Medicaid GME payments by states to teaching programs included funds from federal and state sources and payments made under FFS and managed care. Payments included state-reported and any consultant-estimated amounts. (For 2018, there were no consultant-estimated amounts.) Total payment amounts do not reflect the precise total of individual state amounts because of rounding.

^cFor the survey years shown, the following states reported a total GME payment amount but provided no specific breakdown of amounts paid under FFS or managed care for all or some as noted of their GME payments. GME payment amounts from these states were not included in the calculation of the reported FFS/MC percentages as follows: 2018: GA (some), IL (some), MD, TX (some); 2015: AZ, FL, GA (some), MD, MS (some), OH, TX (some); 2012: AZ, CO, HI, MD, OH; 2009: AZ, CO, HI, IN (some), MD, NJ (some), OH, OR (some); 2005: AZ, CO, FL, IN (some), KS, MD, MA (some), MI, NJ (some), OH, OR (some), RI (some), VA, WA (some).

^dThe 10 states with the largest number of medical residents varies slightly by survey year, and not all of these states paid for GME under Medicaid in each year. States not making Medicaid GME payments for the years noted include the following: CA in 2012, 2015, 2018; IL in 2002, 2005, 2009, 2012; MA in 2009, 2012, 2015, 2018; and TX in 2005, 2009, 2012.

states make GME payments directly to teaching programs or indirectly as part of the capitation rates paid to risk-based managed care organizations.^{4,15} Managed care organizations are not bound to distribute GME payments to teaching hospitals unless required by their state Medicaid program. Six of the 13 states that included GME payments in their managed care organization rates in 2018 required managed care organizations to pass these payments on to teaching hospitals; the remaining states assumed that managed care organizations distributed these payments to hospitals.⁴ Figure A (available as a supplement to the online version of this article at <http://www.ajph.org>) illustrates how Medicaid GME payments are financed and distributed.

CONCERNS AND REMEDIES

Despite the notable growth in Medicaid GME payments and the adoption of innovative GME payment approaches by several states over the past 20 years, the federal government, states, and teaching hospitals now face questions (similar to those directed at Medicare) about the lack of information and accountability, both financial and social, concerning these GME payments.

Incomplete Information

Publicly available information on Medicaid GME payments is often deficient or nonexistent. Medicaid GME payment data collected by CMS is generally acknowledged to be lacking, as (1) not all states provide the requested information, (2) states do not separately report GME payments included in FFS hospital base rates (supplemental GME payments are commonly reported), and (3) state GME

payments made under managed care typically are not reported.^{2,4,11–13}

Moreover, in some states, policymaker and researcher efforts to obtain and evaluate information on GME payments is problematic. For example, the amount of Medicaid GME payments is difficult to quantify precisely in a few states.⁴ Also, most states lack adequate data on the physician workforce implications associated with their Medicaid GME payments (e.g., number and specialty of residents supported, proportion of these residents who go on to serve Medicaid beneficiaries when they enter independent practice).^{2,11,16}

To remedy this data inadequacy, CMS should institute stricter requirements for state Medicaid programs to collect and submit complete and accurate data on GME supplemental payments currently requested as well as GME payments included in FFS hospital base rates and made under managed care. In addition, CMS should require states to collect physician workforce data important to policymakers on their Medicaid GME payments.

Insufficient Financial Accountability

Although Medicaid programs employ a formulaic approach to pay teaching programs for GME,⁴ the means by which hospitals distribute these public funds lacks transparency and is not widely understood by states and affiliated residency programs. In most states, Medicaid GME payments go directly to teaching hospitals and not to the residencies they sponsor.⁴ For many residency programs, this policy obstructs their knowledge of the amount and flow of these payments or whether they even receive such payments.¹⁷ Typically, hospitals distribute GME payments through a general revenue fund with no restriction

that these payments must support only GME. Consequently, this imprecise method of cost accounting makes the benefits to GME of these payments challenging to quantify.^{17,18} Also, there are questions about whether the single amount that most Medicaid programs pay hospitals for resident training accurately reflects the hospital's GME costs when the cost of training is thought to differ by a resident's specialty, year of training, site of training, program size, and other factors.^{2,11,19} In 2018, only one state Medicaid program reported using a payment method that recognizes one or more of these factors.⁴

Teaching programs also have little incentive to keep detailed records of GME revenues and expenses because state and CMS Medicaid payment regulations do not require them to collect and report this information. In 2018, just 14 states required teaching programs to routinely report their allowable direct GME costs; but 21 states noted that they use other sources to obtain this information. In addition, only 14 states said that they routinely audit their GME payments to teaching programs.⁴ Even when these GME costs and audit findings are reported to CMS, the federal government is not required to use the information for oversight or evaluation except in limited cases.²

To improve financial accountability, federal and state regulations should mandate that state Medicaid programs routinely collect detailed records of GME revenues and expenses from teaching hospitals and other entities receiving their GME payments. Moreover, states should be required to regularly audit teaching hospitals to identify or verify the Medicaid GME payments hospitals pass on to residency programs and the number of residents these payments support. In addition, CMS should be compelled to use these audits for

wide-ranging oversight and evaluation. Finally, Medicaid programs should employ a GME payment methodology that recognizes documented cost differences among residencies based on specialty and other variables.

Lack of Social Accountability

Despite Medicaid programs' reported intentions of having GME payments help address their state's health workforce needs, there is no known evidence that such expectations have made any difference. Evidence is lacking because most Medicaid programs do little or nothing to actually track and report the societal value of the public's investment in GME, in part because their state legislatures and CMS do not obligate them to do so. In 2018, just three Medicaid programs stated that they document and report the impact of their GME payments on the state's health care workforce as their state requires.⁴ In addition, CMS does not require states or teaching programs to report any information about the number and characteristics of physician residents that Medicaid GME payments support.²

Nevertheless, most Medicaid programs are unable to document the impact of their GME payments principally because they have not defined clear and explicit goals associated with these payments and the specific, measurable outcomes taxpayers can expect from the payments.⁴ The lack of such goals and performance measures, coupled with a dearth of supporting data, limits efforts by state and federal policymakers to determine whether Medicaid GME payments have made a social difference and what initiatives are needed to enhance the outcomes of these payments.^{2,16}

To make Medicaid GME payments more socially accountable, CMS should require states to define explicit workforce goals and performance measures pertaining to their payments and routinely collect supporting data useful to state and federal policymakers for evaluating the effects of these payments on the state's physician workforce supply and distribution. *AJPH*

CORRESPONDENCE

Correspondence should be sent to Thomas M. Henderson, MSPH, MAMC, Adjunct Faculty, Department of Health Administration and Policy, College of Health and Human Services, George Mason University, 4400 University Dr, Fairfax, VA 22030 (e-mail: thender1@gmu.edu).

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CONFLICTS OF INTEREST

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